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REPORT

NAME: TEST PATIENT KAPPA **AGE/SEX: 55 / F**
REFD. BY: TEST REFERRING DOCTOR **DATE: 15/10/2026**
UHID: TEST-010 **ACCESSION: ACC-MRI-LS-WHOLE-002**

MRI LUMBOSACRAL SPINE WITH WHOLE SPINE SCREENING

This is a limited screening examination of the thoracic and cervical spine. Dedicated imaging is recommended for detailed evaluation.

CLINICAL HISTORY:

Chronic back pain with multilevel symptoms

TECHNIQUE:

Multiplanar, multisequence MRI of the lumbosacral spine was performed without intravenous contrast. Limited screening sequences of the entire spine were also obtained.

MEASUREMENTS:

Measurement	Value
L1-L2 Canal AP	14.2 mm
L2-L3 Canal AP	13.5 mm
L3-L4 Canal AP	12.8 mm
L4-L5 Canal AP	11 mm
L5-S1 Canal AP	12 mm

FINDINGS:

LUMBOSACRAL SPINE: Loss of normal lumbar lordosis. Multilevel degenerative disc disease is noted. At L3-L4, there is a diffuse disc bulge causing mild thecal sac indentation. At L4-L5, there is a posterior disc protrusion measuring approximately 3 mm causing moderate central canal narrowing. At L5-S1, there is disc desiccation with a small posterior disc protrusion. Mild facet arthropathy is present at L4-L5 and L5-S1. The conus medullaris terminates at L1 level and demonstrates normal signal intensity.

WHOLE SPINE SCREENING: Limited evaluation demonstrates multilevel degenerative changes in the cervical spine. At C5-C6, there is a disc osteophyte complex causing mild central canal narrowing on the screening sequences. The visualized thoracic spine is unremarkable. The visualized spinal cord appears normal in caliber within the limitations of the screening examination.

IMPRESSION:

1. **Loss of lumbar lordosis.**
2. **Multilevel lumbar degenerative disc disease:**
3. **- L3-L4 diffuse disc bulge with mild thecal sac indentation**
4. **- L4-L5 posterior disc protrusion with moderate central canal narrowing**
5. **- L5-S1 disc desiccation with small posterior disc protrusion**
6. **Mild facet arthropathy at L4-L5 and L5-S1.**
7. **Limited cervical spine screening: multilevel degenerative changes with C5-C6 disc osteophyte complex causing mild central canal narrowing.**

RECOMMENDATION:

Dedicated cervical spine MRI recommended for detailed evaluation. Clinical correlation for lumbar radiculopathy.

Dr. Sugandha Priyadarshini
MD (Radiodiagnosis & Medical Imaging)

Radiological diagnosis is not always conclusive & often vary with clinical course of the disease or response to treatment. This report is not for medico-legal purpose.